ERYCIN®
FILM COATED TABLET 250MG

Ingredient(s):
Each tablet contains:
Erythromycin stearate (equivalent to erythromycin …...................………………...................… 250mg)

Pharmacology (Summary of Pharmacodynamics and Pharmacokinetics):
Pharmacodynamics
Erythromycin is produced by a strain of *Streptomyces erythraeus* and belongs to the macrolide group of antibiotics. Erythromycin binds to the 50S ribosomal subunits of susceptible bacteria and suppresses protein synthesis.
Erythromycin has good activity against gram-positive and gram-negative bacteria, and other infectious agents, including *Chlamydia trachomatis,* mycoplasmas *(Mycoplasma pneumoniae and Ureaplasma urealyticum),* and spirochetes *Treponema pallidum* and *Borrelia species).*
Erythromycin has good activity against *Streptococcus pneumoniae, S.pyogenes* (group A beta-hemolytic streptococci), and *Staphylococcus aureus.*
Erythromycin also has good activity against certain gram-negative bacteria, including *Legionella pneumophila, Campylobacter jejuni,* and *Bordetella pertussis,* and somewhat lower activity against *Haemophilus influenzae.* There is activity against some gram-negative anaerobs, but most strains of *Bacteroids fragilis* are resistant.
Pharmacokinetics
Orally administered erythromycin are readily and reliably absorbed. Comparable serum levels of erythromycin are achieved in the fasting and nonfasting states. Erythromycin diffuses readily into most body fluids. Only low concentrations are normally achieved in the spinal fluids, but passage of the drug across the blood-brain barrier increases in meningitis. In the presence of normal hepatic function, erythromycin is concentrated in the liver and excreted in the bile; the effect of hepatic dysfunction on excretion of erythromycin by the liver into the bile is not known. Less than 5% of the orally administered dose of erythromycin is excreted in active form in the urine. Erythromycin crosses in placental barrier and is excreted in breast milk.

Indication(s):
Upper and lower respiratory tract infections, skin and soft tissues infections, ear and oral infections.

Dosage and Administration:
Optimum blood levels are obtained when doses are given on an empty stomach or immediately before meals.

Adults: For mild to moderately severe infections, the usual adult dose is 1-4 g/day given in equal amounts, either twice daily (500mg every 12hrs), or 4 times daily (250mg every 6 hrs). In very severe infections, the dosage may be increased up to 4g per day, given in divided dose.

Children: Basic recommendation ranges from 30-50 mg/kg/day or more, depending on the severity of the infection; these amounts are administered in divided doses at 6- or 12-hr intervals.

In streptococcal infections, a therapeutic dosage of the erythromycin should be administered for at least 10 days. For continuous prophylaxis of streptococcal infections in persons with a history of rheumatic disease, the dose is 250mg twice a day.

Route of Administration: Oral

Contraindications:
Patients with known hypersensitivity to this antibiotic.

Precaution(s) / Warning(s):
1. Erythromycin is principally excreted by the liver. Caution should be exercised in administering the antibiotic to patients with impaired hepatic function. There have been reports of hepatic dysfunction, with or without jaundice, occurring in patients receiving oral erythromycin products.

2. During prolonged or repeated therapy, there is a possibility of overgrowth of nonsusceptible bacteria or fungi. If such infections occur, the drug should be discontinued and appropriate therapy instituted.

3. Areas of localized infection may require surgical drainage in addition to antibiotic therapy.

4. Use in pregnancy and lactation: The safety of erythromycin for use during pregnancy has not been established. Erythromycin crosses the placental barrier. Erythromycin also appears in breast milk.

5. Occasional case reports of cardiac arrythmias, e.g. ventricular tachycardia have been documented in patients receiving erythromycin therapy. There have been isolated reports of other cardiovascular symptoms, e.g. chest pain, dizziness and palpitations; however, a cause and effect relationship has not been established.

There have been reports of infantile hypertrophic pyloric stenosis (IHPS) occurring in infants following erythromycin therapy. In one cohort of 157 newborns who were given erythromycin for pertussis prophylaxis, seven neonates (5%) developed symptoms of non-bilious vomiting or irritability with feeding and were subsequently diagnosed as having IHPS requiring surgical pyloromyotomy. Since erythromycin may be used in the treatment of conditions in infants which are associated with significant mortality or morbidity (such as pertussis or chlamydia), the benefit of erythromycin therapy needs to be weighed against the potential risk of developing IHPS. Parents and caregivers should be informed to contact their physician if vomiting and/ or irritability with feeding occurs. In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCARs) [e.g. Stevens-Johnson Syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP) , Erycin Film Coated Tablet 250mg should be discontinued immediately and appropriate treatment should be urgently initiated.

Side Effect(s) / Adverse Reaction(s):
1. The most frequent adverse effects of erythromycin preparations are gastrointestinal, e.g. abdominal cramping and discomfort, and are dose-related. Nausea, vomiting and diarrhea occur infrequently with usual oral doses. Pseudomembranous colitis has been rarely reported in association with erythromycin therapy.

2. Allergic reactions ranging from urticaria and mild skin eruptions to anaphylaxis have occurred.

3. There have been isolated reports of reversible hearing loss occurring chiefly in patients with renal

insufficiency and in patients receiving high doses of erythromycin.
4. There have been isolated reports of transient central nervous system side effects including confusion, hallucinations, seizures and vertigo; however a cause and effect relationship has not been established.

Postmarketing Experience:
Gastrointestinal Disorders: infantile hypertrophic pyloric stenosis

Skin and Subcutaneous Tissue Disorders:
Frequency not known: severe cutaneous adverse reactions (SCARs) including Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP)

Prenancy and lactation:
Use in pregnancy and lactation: The safety of erythromycin for use during pregnancy has not been established. Erythromycin crosses the placental barrier. Erythromycin also appears in breast milk. Erythromycin crosses the placental barrier and is excreted in breast milk.

Drug Interactions:
The use of erythromycin in patients concurrently taking drugs metabolised by the cytochrome P-450 system may be associated with elevations in serum erythromycin with carbamazepine, cyclosporine, hexobarbital and phenytoin. Serum concentrations of drugs metabolised by the cytochrome P-450 system should be monitored closely in patients concurrently receiving erythromycin.

Drug-lab interaction: Erythromycin interferes with the fluorometric determination of urinary catecholamines.

Prolongation of the QT interval and ventricular tachycardia, including torsades de pointes, have been reported in some patients receiving astemizole or terfenadine concomitantly with erythromycin or the structurally related macrolide troleandomycin.

Recent data from studies of erythromycin reveal that its use in patients who are receiving high doses of theophylline may be associated with an increase of serum theophylline toxicity and/or elevated serum theophylline levels, the dose of theophylline should be reduced while the patient is receiving concomitant erythromycin therapy.

Concomitant administration of erythromycin and digoxin has been reported to result in elevated digoxin serum levels. There have been reports of increased anticoagulant effects when erythromycin and oral anticoagulants were used concomitantly.

Concurrent use of erythromycin and ergotamine or dihydroergotamine has been associated in some patients with acute ergot toxicity characterized by severe peripheral vasospasm and dysesthesia.

Erythromycin has been reported to decrease the clearance of triazolam and thus may increase the pharmacologic effect of triazolam.

Patients receiving concomitant lovastatin and erythromycin should be carefully monitored; cases of rhabdomylosis have been reported in seriously ill patients.

Symptoms and Treatment for Overdosage, and Antidote(s):
In case of overdosage, erythromycin should be discontinued. Overdosage should be handled with the prompt elimination of unabsorbed drug and all other appropriate measures. Erythromycin is not removed by peritoneal dialysis or haemodialysis.

Shelf-Life:
3 years from the date of manufacture.

Storage Condition(s):
Store at temperature below 30ºC. Protect from light and moisture.

Product Description and Packing(s):
A red color film coated round tablet, one side impressed with a score and the other side impressed with "YSP"
Blister packing of 10's x 10 and 10's x 100.

Y. S. P.®

Manufacturer and Product Registration Holder:
Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)
Lot 3, 5, 7, 12 & 14, Jalan P/7, Section 13,
Kawasan Perindustrian Bandar Baru Bangi,
43000 Kajang, Selangor, Malaysia.
Ordering Line: 1 800 88 3027
Product Info: 1 800 88 3679

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